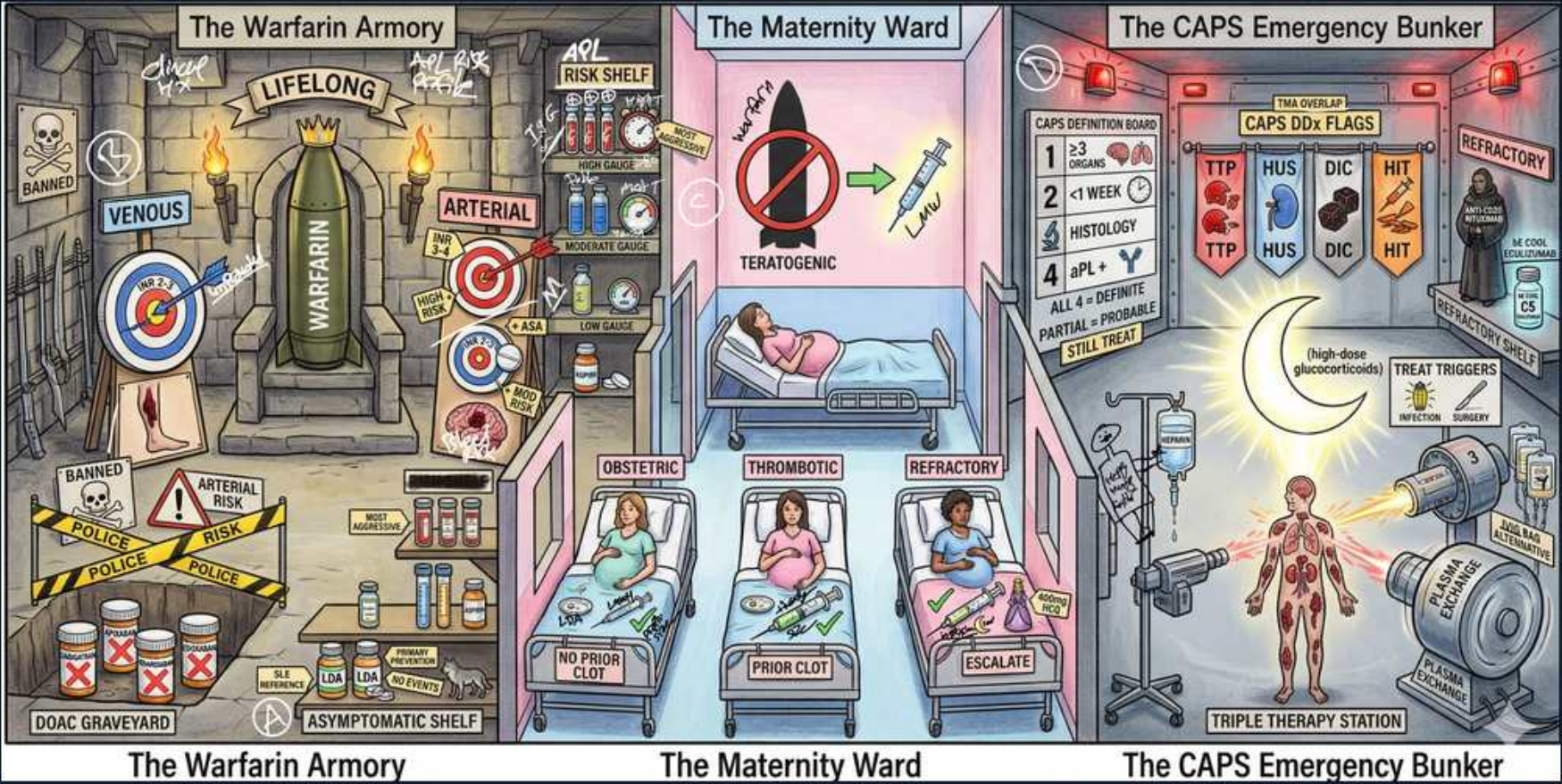




1. Warfarin throne (first-line)

WHAT YOU SEE

Giant warfarin bullet on a throne in 'The Warfarin Armory'.

WHAT IT ENCODES

Warfarin (a vitamin-K antagonist) is first-line for thrombotic APS because it provides durable anticoagulation that DOACs cannot reliably match in this antibody-driven hypercoagulable state. Standard target INR is 2-3 after a venous event; warfarin inhibits VKORC1, depleting factors II, VII, IX, X and proteins C and S. Always bridge with heparin/LMWH at initiation to cover the transient protein-C-driven procoagulant window.

BOARD TRAP

DOACs (rivaroxaban, apixaban) are the wrong answer for thrombotic APS — the TRAPS trial showed significantly higher arterial recurrence with rivaroxaban, especially in triple-positive (lupus anticoagulant + anticardiolipin + anti-β2GP1) patients.

2. Lifelong banner

WHAT YOU SEE

'LIFELONG' banner over the armory.

WHAT IT ENCODES

Anticoagulation after a thrombotic APS event is indefinite/lifelong, unlike a provoked VTE which stops at 3-6 months. The persistent antiphospholipid antibodies represent an ongoing prothrombotic driver, so recurrence risk stays high if therapy is withdrawn. Mnemonic: APS clot = 'A Permanent Script' for anticoagulation.

BOARD TRAP

Stopping anticoagulation at a fixed 3-6 months (the rule for a provoked clot) is the classic trap — in confirmed APS the antibodies persist, so the clot is effectively 'unprovoked' and treatment continues indefinitely.

3. Venous target (INR 2-3)

WHAT YOU SEE

'VENOUS' dartboard target.

WHAT IT ENCODES

After a venous thrombosis, target INR is 2-3 with warfarin — the same intensity used for standard VTE. Higher-intensity warfarin (INR 3-4) was studied and showed NO benefit over INR 2-3 for venous disease while increasing bleeding. So for venous APS, conventional intensity wins.

BOARD TRAP

Reflexively pushing INR to 3-4 for any APS clot is wrong for venous events — trials (Crowther 2003) found high-intensity warfarin gave no extra protection and more bleeding for venous disease.

4. Arterial / higher-intensity risk shelf

WHAT YOU SEE

'ARTERIAL' target and '≥3 RISK SHELF'.

WHAT IT ENCODES

Arterial APS events (stroke, MI) often warrant either higher-intensity warfarin (target INR ~3, range 3-4) or warfarin (INR 2-3) PLUS low-dose aspirin. Triple-positivity and recurrent thrombosis push toward more aggressive regimens. The arterial bed is more prone to recurrence, so single-agent low-intensity therapy is frequently inadequate.

BOARD TRAP

Treating an arterial APS stroke with antiplatelet monotherapy (aspirin alone) is a common wrong answer — anticoagulation, often intensified or combined with aspirin, is needed; aspirin alone underprotects against recurrent arterial thrombosis.

5. DOAC graveyard (banned)

WHAT YOU SEE

Crossed-out pill bottles in 'DOAC GRAVEYARD', police tape.

WHAT IT ENCODES

DOACs (rivaroxaban, apixaban, dabigatran, edoxaban) are avoided/contraindicated in high-risk thrombotic APS. The 2019 EULAR guidance recommends against DOACs in triple-positive patients and after arterial events. Mechanism rationale: factor Xa/IIa inhibition appears insufficient against the multi-hit antibody-mediated thrombogenesis.

BOARD TRAP

Switching a stable APS patient to a DOAC for 'convenience' or to avoid INR monitoring is the signature board trap — especially in triple-positive or arterial disease, where DOACs raise recurrence.

6. Asymptomatic shelf

WHAT YOU SEE

'ASYMPTOMATIC SHELF' lower right of armory.

WHAT IT ENCODES

Asymptomatic aPL carriers (positive antibodies, no clot) may receive low-dose aspirin 81 mg for primary prophylaxis, especially with a high-risk profile (triple-positive, persistent lupus anticoagulant) or coexisting SLE, where hydroxychloroquine adds protection. Risk-factor modification (avoid estrogen, treat HTN/dyslipidemia) is essential.

BOARD TRAP

Putting an asymptomatic aPL carrier on full therapeutic anticoagulation is over-treatment — they have never clotted, so the benefit-to-bleed ratio favors at most low-dose aspirin, not warfarin.

7. Warfarin teratogenic (no in pregnancy)

WHAT YOU SEE

Warfarin bullet crossed out, 'TERATOGENIC' with syringe.

WHAT IT ENCODES

Warfarin crosses the placenta and is teratogenic — first-trimester exposure causes warfarin embryopathy (nasal hypoplasia, stippled epiphyses/chondrodysplasia punctata) and later CNS/bleeding complications. It must be stopped before conception or as soon as pregnancy is confirmed and replaced with LMWH, which does not cross the placenta. Mnemonic: 'Warfarin Wrecks the Womb.'

BOARD TRAP

Continuing warfarin into pregnancy is the trap — it causes fetal warfarin syndrome; the correct move is to switch to heparin/LMWH (heparins are large and do not cross the placenta).

8. Obstetric bed — no prior clot

WHAT YOU SEE

Bed 'OBSTETRIC / NO PRIOR CLOT'.

WHAT IT ENCODES

Obstetric APS without prior thrombosis (recurrent miscarriage/late fetal loss/severe pre-eclampsia) is treated with PROPHYLACTIC-dose LMWH (e.g., enoxaparin 40 mg daily) PLUS low-dose aspirin 81 mg, ideally started preconception/early pregnancy. This combination improves live-birth rates by preventing placental microthrombosis.

BOARD TRAP

Aspirin alone is insufficient for obstetric APS — the heparin-plus-aspirin combination is what raises live-birth rates; choosing aspirin monotherapy is the undertreatment trap.

9. Thrombotic bed — prior clot

WHAT YOU SEE

Bed 'THROMBOTIC / PRIOR CLOT'.

WHAT IT ENCODES

A pregnant APS patient with a prior thrombosis needs THERAPEUTIC (full-dose) LMWH (e.g., enoxaparin 1 mg/kg BID) plus low-dose aspirin throughout pregnancy, then transition back to warfarin postpartum (warfarin is safe in breastfeeding). They were on lifelong anticoagulation already, so pregnancy converts warfarin to full-dose LMWH, not prophylactic dosing.

BOARD TRAP

Dropping a prior-clot patient to prophylactic LMWH during pregnancy is the trap — a history of thrombosis demands therapeutic dosing; prophylactic dosing leaves them under-anticoagulated.

10. Refractory bed — escalate

WHAT YOU SEE

Bed 'REFRACTORY / ESCALATE'.

WHAT IT ENCODES

Refractory obstetric APS (pregnancy loss despite LMWH + aspirin) is escalated by adding hydroxychloroquine (immunomodulatory, lowers antibody titers and stabilizes placental function) and/or low-dose prednisone in the first trimester; IVIG/plasmapheresis are last-resort. HCQ is increasingly favored adjunctively in APS pregnancies.

BOARD TRAP

Abandoning anticoagulation or jumping straight to high-dose chronic steroids is wrong — escalation builds ON the LMWH/aspirin backbone with HCQ first, not instead of it.

11. CAPS definition board

WHAT YOU SEE

'CAPS DEFINITION BOARD': ≥ 3 organs, < 1 week, histology, aPL+.

WHAT IT ENCODES

Catastrophic APS (CAPS, 'Asherson syndrome') = involvement of ≥ 3 organs/systems developing simultaneously over < 1 week, with histopathologic confirmation of small-vessel occlusion and laboratory aPL positivity. It is rare ($< 1\%$ of APS) but carries $\sim 30\text{-}50\%$ mortality. Kidneys, lungs, brain, heart, and skin are commonly hit.

BOARD TRAP

CAPS is a microvascular small-vessel thrombotic storm — confusing it with the usual single large-vessel APS event (one DVT/stroke) misses the diagnosis; CAPS is the 'all organs at once' catastrophe.

12. CAPS DDx flags (TTP/HUS/DIC/HIT)

WHAT YOU SEE

Flags 'TTP, HUS, DIC, HIT' as differentials.

WHAT IT ENCODES

CAPS must be distinguished from other thrombotic microangiopathies: TTP (ADAMTS13 $< 10\%$, treat with PLEX), HUS (Shiga toxin/complement, schistocytes), DIC (consumptive coagulopathy with low fibrinogen and prolonged PT/PTT), and HIT (platelet drop 5-10 days after heparin, positive PF4 antibody). aPL positivity plus multi-organ small-vessel thrombi points to CAPS.

BOARD TRAP

Mislabeling CAPS as DIC is a key trap — in DIC fibrinogen falls and bleeding dominates, whereas CAPS is primarily a thrombotic storm with aPL positivity; the wrong label delays immunosuppression and plasma exchange.

13. Triple therapy station

WHAT YOU SEE

IV pole + apheresis machine, 'TRIPLE THERAPY STATION', high-dose glucocorticoids.

WHAT IT ENCODES

First-line CAPS therapy is TRIPLE therapy: therapeutic anticoagulation (IV heparin) + high-dose glucocorticoids (e.g., methylprednisolone 1 g IV pulse x3 days) + plasma exchange and/or IVIG. Refractory cases add rituximab or eculizumab (complement blockade). The combination targets clot, inflammation, and circulating antibodies simultaneously.

BOARD TRAP

Anticoagulation alone is inadequate in CAPS — the trap is treating it like a routine clot; you must add immunosuppression (steroids) plus PLEX/IVIG to halt the cytokine-and-antibody-driven storm.

14. Treat triggers

WHAT YOU SEE

'TREAT TRIGGERS' with infection/surgery icons.

WHAT IT ENCODES

CAPS is precipitated in $\sim$ half of cases by an identifiable trigger — most commonly infection, but also surgery/trauma, anticoagulation withdrawal, malignancy, and pregnancy/puerperium. Aggressively identifying and treating the precipitant (e.g., antibiotics for infection) is part of management alongside triple therapy.

BOARD TRAP

Treating only the thrombosis while ignoring an underlying infection or subtherapeutic/withdrawn anticoagulation misses the driver — failure to remove the trigger lets the storm continue despite anticoagulation.